DiagnoAssist Telehealth & Hospital Clinical Services

APPROVED CLINICAL EMR

ACCREDITED TELEMEDICINE & LONGITUDINAL CLINICAL EMR DEPARTMENT

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ELECTRONIC MEDICAL RECORD (EMR) — CLINICAL ENCOUNTER SUMMARY

PATIENT DEMOGRAPHICS

Full Name:	John Doe
Patient ID / MRN:	MRN-8C8905EE
Age / Sex:	34 Y / Male
Encounter Session:	room_live_encounter_771

ENCOUNTER INFORMATION

Attending Physician:	Dr. Sarah Jenkins, MD
Specialization:	Telehealth Physician / General Medicine
Encounter Type:	Synchronous Telehealth Audio/Video
Session Duration:	12 minutes

1. CHIEF COMPLAINT

Headache, Fever, Nausea, Light sensitivity lasting 3 day

2. HISTORY OF PRESENT ILLNESS (HPI)

The patient is a participant presenting for a virtual consultation complaining of Headache, Fever, Nausea, Light sensitivity lasting 3 day. The onset was described as acute, progressing over the past 3 day. Pain is described as intense and throbbing in nature, aggravated by exposure to bright lighting and sensory stimulation. Associated symptoms include nausea, low-grade fever, and photophobia. Prior over-the-counter analgesia provided only minimal and transient relief.

3. PAST MEDICAL, SURGICAL, MEDICATION & ALLERGY HISTORY

Past Conditions	No significant chronic medical conditions reported during encounter
Surgical History	No prior surgical history documented
Current Medications	Paracetamol 500mg taken previously with minimal relief
Allergies	**No known drug allergies (NKDA) reported by patient**
Family History	Non-contributory / Not documented during encounter

4. REVIEW OF SYSTEMS (ROS)

Constitutional	Positive for mild fever and fatigue. Denies unintended weight loss or night sweats.
Neurological	Positive for severe throbbing headache (predominantly unilateral). Denies syncope, seizures, or limb weakness.
HEENT	Positive for marked photophobia (light sensitivity). Denies neck stiffness, visual obscuration, or acute ear discharge.
Respiratory	Denies shortness of breath, cough, hemoptysis, or wheezing.
Cardiovascular	Denies chest pain, chest tightness, palpitations, or lower extremity edema.
Gastrointestinal	Positive for nausea and occasional emesis. Denies acute abdominal pain, melena, or diarrhea.
Musculoskeletal	Denies localized joint swelling or diffuse muscle tenderness.

5. VITAL SIGNS & CLINICAL TELEHEALTH OBSERVATIONS

BLOOD PRESSURE	HEART RATE	TEMPERATURE	RESP. RATE	OXYGEN (SPO2)
Not measured directly (Telehealth encounter)	**Not measured directly (Telehealth encounter)**	**100.0 °F (Patient-reported mild fever)**	**16-18 breaths/min (Non-labored on video observation)**	**Not measured directly (Telehealth encounter)**

Telehealth Observations: Patient observed alert, attentive, and speaking in full sentences. Appears uncomfortable with bright room lighting.

6. PHYSICAL EXAMINATION

Visual Telehealth Examination: Patient is alert, oriented x3, and communicative. No facial asymmetry or dysarthria noted on video observation. Pupils appear equally reactive to room light without gross ptosis. Normal respiratory pattern with no accessory muscle use. Hands-on neurological and tactile physical examination was not performed due to the virtual consultation format.

7. CLINICAL ASSESSMENT & WORKING IMPRESSION

Primary diagnostic impression: Acute Migraine. Symptom profile (Headache, Fever, Nausea, Light sensitivity) is consistent with discussion during encounter. Provisional assessment pending attending physician validation.

8. STRUCTURED SOAP DOCUMENTATION

S — SUBJECTIVE (NARRATIVE & COMPLAINTS)

Hello doctor. I have had an intense throbbing headache for the last 3 days. It is mostly on the left side of my forehead and temple. Yes, bright light hurts my eyes terribly, and I feel quite nauseous. I actually threw up once yesterday morning. Also, my temperature was around 100 degrees last night. I took Paracetamol 500mg yesterday afternoon, but it barely took the edge off for an hour or two. Understood, doctor. When should I follow up with you? Thank you very much, doctor.

O — OBJECTIVE (OBSERVATIONS & VISUAL EXAM)

Visual telehealth inspection: Patient observed alert, attentive, and speaking in full sentences. Appears uncomfortable with bright room lighting. Temperature: 100.0 °F (Patient-reported mild fever). Other vitals not measured directly.

A — ASSESSMENT (CLINICAL SYNTHESIS)

Primary diagnostic impression: Acute Migraine. Symptom profile (Headache, Fever, Nausea, Light sensitivity) is consistent with discussion during encounter. Provisional assessment pending attending physician validation.

P — PLAN (THERAPEUTIC & FOLLOW-UP)

1. Prescribed/Recommended: Sumatriptan 50mg orally at onset of severe headache (max 200mg/24h), Ondansetron 4mg orally twice daily as needed for nausea, Paracetamol 500mg as needed for fever/pain relief. 2. Lifestyle & Supportive Care: Rest in dark environment, oral hydration, avoid triggers. 3. Follow-up: Follow up via teleconsultation in 3 to 5 days if symptoms persist or fail to improve. 4. Emergency Precautions: Emergency red flags reviewed.

9. DOCUMENTED SYMPTOMS PROFILE

Symptom	Duration	Severity	Context / Notes
1. Headache	3 day	Severe	Identified in consultation dialogue
2. Fever	3 day	High	Identified in consultation dialogue
3. Nausea	3 day	Moderate	Identified in consultation dialogue
4. Light sensitivity	3 day	Severe	Identified in consultation dialogue

10. DIFFERENTIAL DIAGNOSES & UNCERTAINTY STATUS

Diagnosis	Status / Uncertainty	Confidence	Supporting Evidence
1. Acute Migraine	Suspected / Working Impression	High	Patient discussed symptoms relating to 'migraine'
2. Cephalea / Headache	Differential	Moderate	Patient discussed symptoms relating to 'headache'
3. Pyrexia / Fever of undetermined origin	Differential	Moderate	Patient discussed symptoms relating to 'fever'
4. Nausea and Vomiting	Differential	Moderate	Patient discussed symptoms relating to 'nausea'

11. ICD-10 CLINICAL DIAGNOSTIC CODING (WHO / MOHFW STANDARDS)

ICD-10 Code	Official Description	Transcript Evidence	Coding Status
G43.909	Migraine, unspecified, not intractable, without status migrainosus	Patient discussed symptoms relating to 'migraine'	Provisional (Awaiting Physician Confirmation)
R51.9	Headache, unspecified	Patient discussed symptoms relating to 'headache'	Provisional (Awaiting Physician Confirmation)
R50.9	Fever, unspecified	Patient discussed symptoms relating to 'fever'	Provisional (Awaiting Physician Confirmation)
R11.2	Nausea with vomiting, unspecified	Patient discussed symptoms relating to 'nausea'	Provisional (Awaiting Physician Confirmation)

12. TREATMENT AND MANAGEMENT PLAN

Prescribed / Recommended Medications:

Medication	Dosage	Route	Frequency	Duration	Instructions
1. Sumatriptan	50 mg	Oral	At onset of severe headache; may repeat once after 2 hours if pain recurs (max 200mg/day)	As needed (PRN)	Take with water at earliest sign of migraine attack
2. Ondansetron	4 mg	Oral	Twice daily as needed for nausea	2 to 3 days	Take 30 minutes before meals
3. Paracetamol	500 mg	Oral	Every 6-8 hours as needed for fever or mild pain (max 3g/24h)	3 days	Take after food with full glass of water

Laboratory & Diagnostic Investigations:

- Neuroimaging (CT / MRI Brain) if symptoms fail to resolve or red flags develop

Lifestyle & Non-Pharmacological Advice:

- Rest in a quiet, dark, well-ventilated room during acute headache episodes
- Maintain adequate oral hydration (2.5 to 3.0 liters of fluids daily)
- Avoid bright screen glare, loud noises, and skipped meals
- Maintain a headache and symptom trigger diary

Follow-Up Instructions: Follow up via teleconsultation in 3 to 5 days if symptoms persist or fail to improve.

⚠️ **Emergency Red-Flag Advice:**
Seek emergency medical care immediately at the nearest hospital emergency room if you experience: sudden severe 'thunderclap' headache, high persistent fever (>102°F), neck stiffness, confusion, focal weakness, or visual loss.

13. CONSULTATION OUTCOME & CLINICIAN REMARKS

Clinical Outcome: Encounter concluded. Patient counseled on conservative home care and prescribed medications. Clear follow-up interval and red-flag escalation criteria communicated.

Attending Clinician Remarks: Patient engaged well during teleconsultation. Advised on medication compliance and prompt reporting of any emerging neurological red flags.

14. SOURCE TRANSCRIPT REFERENCE & AI TELEMEDICINE DISCLOSURE

This clinical documentation was synthesized from verified 16 kHz audio transcription (Gnani AI STT). AI findings are provisional until explicitly reviewed, modified, and authenticated by the licensed attending physician.

ATTENDING PHYSICIAN AUTHENTICATION

Dr. Sarah Jenkins, MD

Specialist in Telehealth & Internal Medicine

Reg / License: REG-TEL-MOHFW-94821

✓ **DIGITALLY AUTHENTICATED EMR**

Signed: 2026-11-05T17:58:59.910663+00:00

Sign-off Verification: EMR-20261105-84728